



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : انتصار محمد جاب الله السحيمي

العنوان : +++

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 22-09-2026

رقم البطاقة : 27311050103461 رقم التليفون : 01122850948

العمر (تاريخ الميلاد) : 05-11-1973 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6396] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: انتصار محمد جاب الله السحيمي National ID: 27311050103461	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative care with follow-up
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



انتصار
محمد جاب الله السحيمي
٣ عطلة الامل - ش الاكحل - الملاوه
اليساتين - القاهرة

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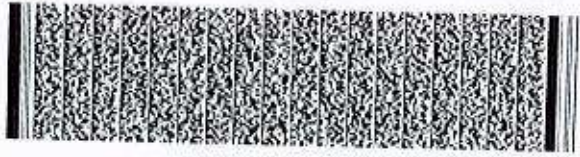
٢٠٢١/٠٣ ٢٧٣١١٠٥٠١٠٣٤٦١



حاصله على ليسانس
دراسات اسلامية وعربية
انثى



البطاقة سارية حتى ٢٠٢٨/٠٣/١١



		جمهورية مصر العربية وزارة الصحة العامة إدارة رئيس الجمهورية لدعم صحة المرأة المصرية		التاريخ 08/09/2026
MDT FORM				
Name:	Intsar Mohamed Gaballah	MRN:	8545	
Age:	53 Y	ID:	27311050103461	
Menopausal Sta:	pre	Phone#:	01106344023	
NRS SCORE:		Marital Status:		
BMI:		Family History:		
Gender:		Reason Of MDT:		
CO Morbidity:	free	OP:		
Surgical History:	nad	ECOG Performance Status:		
			0 1 2 3 4	

Clinical Data:

ER: ☐ 6/8 ☐ 6/8 ☐ 6/8 ☐ 6/8
 PR: ☐ 6/8 ☐ 6/8 ☐ 6/8 ☐ 6/8
 HER2: +2 amp ☐ 30 %

History:

condition started 2024 by met LT breast cancer T3 N1 M1 to lung with multiple pulmonary nodules largest was 8 mm
 revolved tamoxifen 12 wks then maintenance AI + zoledex + herceptin till now
 with regular assessment by CT cap that showing stationary course of pulmonary nodules, and regression of breast mass

Pt reported that she was planned for BSM + surgery but she refused due to being a tailor and fear of lymphedema

recently : by O/E pt has skin nodules over lt breast

bi sonomamography 3-8-2025 :

- UOQ breast and retroareolar still seen saparated dense lesion with pleomorphic microcalcifications in area about 9x9.5x5cm -
- U/S progressive course regarding the previously reported two malignant mass lesions seen -at retro areolar measuring 26x25 mm ?? newly developed - at 3 o'clock zone b measured 36x26mm (previously measured 15x20 mm) Mild skin thick at lower quadrants
- Enlarged one lymph node at level I measures 17x7mm with focal cortical thickening 6m

CT cap 17-6-2026 >> DP of pulmonary nodules 12*10 mm

pt on AI zoledex herceptin since 2024 till now

Sonomamography:**Biopsy/Molecular:****Meta Workup:****Recommendations:**

according to Dr hanan >> for xeloda tykerb

MDT Outcome & Plan

for xeloda tykerb

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
Dr Hanan Ezzat			



Patient Name	: الأستاذة / انتصار محمد جالب الله السحيمي	Request No.	: 13001332
Patient No.	: 27311050103461	Request Date	: 01/10/2023
Path No.	: 3345/23	Verification Date	: 11/10/2023
Sex	: Female	Financial Category	: أورام دار السلام - مبادئ
Age	: 49 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 30/9/2023, 1:40 pm.

Gross Examination:

- Three grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No.
- * Perineural invasion: Positive.
- * Angioinvasion: No.

Diagnosis:

Left breast swelling, Tru-cut biopsy:

Invasive duct carcinoma G II.

For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Heba Hassan.

Verified by :





Patient Name	: الأستاذ / انتصار محمد جاب الله السحيمي	Request No.	: 13001332
Patient No.	: 27311050103461	Request Date	: 01/10/2023
Path No.	: 3345/23	Verification Date	: 28/10/2023
Sex	: Female	Financial Category	: اورام دار السلام - ميانرد
Age	: 49 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

* IPT (Positive control showed appropriate reactivity):

- ER: Strong nuclear reaction in 30% of tumor cells (Allred score 6/8).
- PR: Strong nuclear reaction in 30% tumor cells (Allred score 6/8).
- Her2: Moderate complete membrane reaction in >10% tumor cells (score +2).
- Ki67: 30% nuclear reaction of tumor cells.
- SISH : Amplified with clusters .

Conclusion:

Left breast cancer, Tru-cut biopsy:

Invasive duct carcinoma, Grade II,

IPT:

- ER: Positive (score 6/8).
- PR: Strong (score 6/8).
- Her2: Equivocal (score +2) .
- Ki67: Positive in 30% of tumor cells.
- SISH : Amplified with clusters .

Verified by :





Patient Name الأستاذ / انتصار محمد جاب الله السحيمي
Patient No. 27311050103461
Path No. 3345/23
Sex Female
Age 49 Y
Referring Doc.

Request No. 13001332
Request Date 01/10/2023
Verification Date 28/10/2023
Financial Category اورام دار السلام - مبالغه
Department Out Patient
Registered by Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Signature
Prof.Dr Lobna Omar
Dr Marwa Mahmoud
Dr.Heba Hassan

Verified by :





Exam Information:

<u>Patient Name:</u> Entesar Muhammed	<u>Date of birth:</u> 03/10/1963
<u>Arabic Name:</u> انتصار محمد	<u>Patient Gender:</u> F
<u>Patient RIS ID:</u> 345979	<u>Exam Date:</u> 02/01/2024
<u>Patient HIS ID:</u> 124000127	
<u>Written by:</u> Dr. Hoda Fathy	<u>Approved by:</u> Dr. Hoda Fathy

^{99m}Tc MDP Bone Scan Report (2/1/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP Revealed:

- No definite evidence of distant metastatic deposits could be seen over the surveyed skeleton.
- Degenerative changes over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged

Alaa Elsaeed

Dr/Huda Fathy,MD

مقارنة الماموجرام

NAME: ENTESAR MOHAMED GAB

DATE: 08/01/2026

**FOLLOW UP BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH
HIGH RESOLUTION BREAST SONOGRAPHY REVEALED:**

CLINICAL SUMMARY:

A known case of left carcinoma, under NACT and hormonal therapy, for follow up

FINDINGS:

LEFT BREAST:

- Fairly dense, (ACR B).
- The UOQ is still the seat of a spiculated mass, associated pleomorphic calcifications.
- Complimentary U/S demonstrated the underlying hypoechoic mass located at 3 o'clock paraareolarly, about 6mm deep to the skin, measuring about 2x1.5cm, previously 2x1.8cm.
- No definite pathological axillary nodes are seen in today's study.

RIGHT BREAST:

- Fairly dense, (ACR B).
- No mass, spiculation or suspicious micro calcifications, this was confirmed by an U/S examination.
- Normal skin/nipple/areola complex.
- Non specific axillary nodes are noted.

CONCLUSION:

Left breast (BIRADS 6): rather stationary course of the residual malignant mass as described

Right breast (BIRADS 1): normal study

Best regards,
Dr. Mahi Mokhtar

8/1/2026

**Digitalized Low Dose Soft Tissue Mammography With
Complementary Breast US Examination Of Both Breasts**
S/P NCTH and hormonal ttt for Lt breast cancer

LT breast:

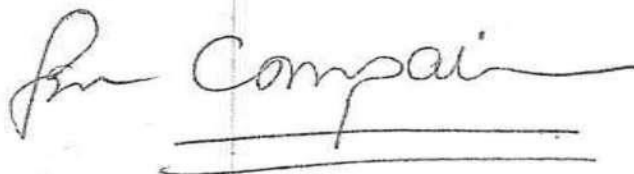
- UOQ residual obscured speculated isodense mass lesion with overlying malignant pleomorphic microcalcification
- On US there is Lt 3 o'clock paraareolar residual irregular ill defined speculated hypoechoic mass lesion with surrounding echogenic dysmoplastic reaction measuring collectively 2 x 1.5 cm with internal microcalcification , another similar smaller nearby satellite is seen measuring 1.1 x 1.2 cm
- No pathological axillary / supraclavicular LNs .

Rt breast:

- ACR B: scattered fibroglandular prechyma
- No spiculated lesions or clustered microcalcification
- On US there is No solid or cystic lesions.
- Normal skin thickness and contour
- Normal nipple areola complex
- Non specific axillary lymph nodes showing preserved fatty hila.(recent post vaccination)

Opinion:

- S/P NCTH and hormonal ttt for Lt breast cancer showing local residual as described (BIRADS 6)
- Normal Rt breast study (BIRADS 1)



*Best regards
Dr. Radwa Mag
Breast Imaging Consultant
NCI, Cairo University*

Patient Name: Entesar Mohamed Gab Allah Alsehim	Date : 22/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 3910/2023	Pat. ID: 42365

POST CONTRAST MULTI-SLICE CT SCAN OF CHEST , ABDOMEN & PELVIS
REVEALED: -

Compared to the last study dated (6/2/2025):

Chest

- **Stationary course** of left breast LOQ irregular speculated mass lesion 1.5 x1 cm with persistent mild diffuse skin thickening.
- **Rather stationary** number and size of the few bilateral small and tiny pulmonary nodules with the largest still seen at superior segment of left lower lung lobe measuring about 8 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen and pelvis:

- The liver is mildly enlarged with diffuse homogenous low parenchymal CT density. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Left renal cortical cyst still noted.
- Normal appearance of the pancreas, aorta, I.V.C and both adrenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- **Still noted:** Bulky uterus with internal IUD .
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

1. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- **Overall stationary course since last dated study.**

Signature Reeham.Khader
25/10/2025 12:02 AM

Patient's Name:	Initisar Mohammed Jab Allah
Date:	2026-06-17
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.

ABDOMEN AND PELVIS REVEALED:

Compared to the previous study dated 22-10-2025, the current study showed:

I- CHEST:

- Progressive course since last study regarding the size of the previously seen left breast lesion measuring 19 x 13 mm with few small newly developed adjacent nodules for sono-mammography.
- Progressive course since last study regarding the size and number of the previously seen bilateral scattered pulmonary nodules largest measuring 12 x 10 mm at the right upper lobe.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes. Left renal cortical cyst.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

Patient's Name:	Initisar Mohammed Jab Allah
Date:	2026-06-17
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

- ❖ Progressive course since last study regarding the left breast lesions and bilateral scattered pulmonary nodules.
- ❖ Stationary course regarding the rest of the study.

Dr.sherif Wagdy

Dr.sherif Wagdy

Patient's Name:

Intisar Mohammed Jab Allah Al-Suhaimi

Date:

2026-08-03

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Clinical summary: Pathologically proven left breast cancer for assessment.

Findings:

Right breast

- Fairly dense breast parenchyma (ACR B)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- U/S No solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast

- UOQ and retroareolar still seen sapculated dense lesion with pleomorphic microcalcifications in area about 9x9.5x5cm
- U/S progressive course regarding the previously reported two malignant mass lesions seen -at retro areolar measuring 26x26 mm ?? newly developed
- at 3 o'clock zone b measured 36x26mm (previously measured 15x20 mm)
- Mild skin thick at lower quadrants
- Enlarged one lymph node at level I measures 17x7mm with focal cortical thickening 6mm .

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 6): progressive course regarding the previously reported malignant process.

Best Regards,

DR. Marwa M.Hassan

Patient's Name:	Initisar Mohammed Jab Allah
Date:	2026-06-17
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.
ABDOMEN AND PELVIS REVEALED:

Compared to the previous study dated 22-10-2025, the current study showed:

I- CHEST:

- Progressive course since last study regarding the size of the previously seen left breast lesion measuring 19 x 13 mm with few small newly developed adjacent nodules for sono-mammography.
- Progressive course since last study regarding the size and number of the previously seen bilateral scattered pulmonary nodules largest measuring 12 x 10 mm at the right upper lobe.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes. Left renal cortical cyst.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

Patient's Name:	Initisar Mohammed Jab Allah
Date:	2026-06-17
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

- ❖ Progressive course since last study regarding the left breast lesions and bilateral scattered pulmonary nodules.
- ❖ Stationary course regarding the rest of the study.

Dr.sherif Wagdy

Dr.sherif Wagdy

Patient's Name:

Intisar Mohammed Jab Allah Al-Suhaimi

Date:

2026-08-03

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Clinical summary: Pathologically proven left breast cancer for assessment.

Findings:

Right breast

- Fairly dense breast parenchyma (ACR B)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
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- U/S progressive course regarding the previously reported two malignant mass lesions seen -at retro areolar measuring 26x26 mm ?? newly developed - at 3 o'clock zone b measured 36x26mm (previously measured 15x20 mm)
- Mild skin thick at lower quadrants
- Enlarged one lymph node at level I measures 17x7mm with focal cortical thickening 6mm .

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 6): progressive course regarding the previously reported malignant process.

Best Regards,

DR. Marwa M.Hassan